

See discussions, stats, and author profiles for this publication at: <https://www.researchgate.net/publication/385552506>

Beyond controversy –International perspectives on sexual and reproductive health education

Article in *Health Education Journal* · November 2024

DOI: 10.1177/00178969241297016

CITATIONS

4

READS

158

4 authors, including:



Deevia Bhana

University of KwaZulu-Natal

211 PUBLICATIONS 3,172 CITATIONS

[SEE PROFILE](#)



Esther Miedema

University of Amsterdam

47 PUBLICATIONS 416 CITATIONS

[SEE PROFILE](#)



Ndumiso Daluxolo Ngidi

University of Fort Hare

27 PUBLICATIONS 264 CITATIONS

[SEE PROFILE](#)

Beyond controversy – International perspectives on sexual and reproductive health education

Health Education Journal

1–9

© The Author(s) 2024

Article reuse guidelines:

sagepub.com/journals-permissions

DOI: 10.1177/00178969241297016

journals.sagepub.com/home/hej

**Deevia Bhana^a, Peter Aggleton^{b,c},
Esther Miedema^d and Ndumiso Daluxolo Ngidi^a**

^aSchool of Education, University of KwaZulu-Natal, Durban, South Africa

^bCentre for Social Research in Health, UNSW Sydney, Sydney, NSW, Australia

^cInstitute for Global Health, University College London, London, UK

^dFaculty of Social and Behavioural Sciences, University of Amsterdam, Amsterdam, The Netherlands

Introduction

The World Health Organization (2006) defines sexual and reproductive health as a condition of overall physical, mental, and social well-being concerning all aspects of the reproductive system. This state of well-being is based on the right of people to engage in sexually satisfying and safe sexual lives alongside the right to determine if, when, and how often to reproduce. Ensuring the realisation of sexual and reproductive health and well-being requires access to education and health care that is effective in engaging with sex as a pleasurable aspect of life while protecting people against unwanted pregnancy, gender and sexual violence, sexually transmitted infections, and risk. Sexual and reproductive health education, despite contestation, remains vital to the realisation of well-being and overall health (Zarocostas, 2024)

Despite its value, education about sexual and reproductive health remains a global challenge. As Aggleton et al. (2024) suggest, the contemporary context is defined by growing conservatism, homophobia, transphobia, and gender wars underpinned by patriarchal norms, colonial legacies and neo-liberal agendas. In many countries, socio-structural inequalities and crises, overburdened healthcare systems, gender and sexual violence, and religious and political polarisations compromise the ability of all people to make decisions about their health and to attain sexual well-being (Starrs et al., 2024). Recent upheavals, including ‘natural’ disasters; wars and civil conflict; the influence of conservative right-wing politics on sexual and reproductive rights including abortion rights; COVID-19; and continued complacency about HIV, have set back education on sexual and reproductive health care and rights (Diniz and Cabral, 2022). These challenges exacerbate existing inequalities and point to health fractures along the lines of region, rural-urban disparities, race, class, ability, gender, sexuality, citizenship, ethnicity and age.

Corresponding author:

Deevia Bhana, School of Education, University of KwaZulu-Natal, Private Bag X54001, Durban, 4000, South Africa.

Email: bhanad1@ukzn.ac.za

Sexual and reproductive health education often clashes with cultural and religious values, leading to significant debates about the kinds of healthcare services and education that should be made available to young people, both in schools and colleges, and in public health settings (Zajacova and Lawrence, 2018). Some argue that the provision of such education undermines traditional values and/or encourages sexual activity, particularly among young people (Nash et al., 2019). The inclusion of topics such as contraception, abortion, sex, sexuality and gender diversity, and sexual pleasure remains controversial, despite the need for education about these important issues. These controversies are also context-dependent, with different regions, communities, and countries adopting diverse approaches to sexual and reproductive health education.

Despite the immense challenges and everyday violence that compromise sexual health and well-being, there are emerging stories of those working in the field who are pushing hard to provide good sexual and reproductive health and education in often difficult circumstances. Beyond the provision of scientifically accurate information, good-quality education on sexual and reproductive health requires a focus on gender and sexuality equality, disability, rights, respect, safety, freedom from violence and discrimination. It thus involves understanding and addressing gender power relations, normative understandings of sex and sexuality, and the ideologies through which masculine entitlements are upheld. Good-quality education for sexual and reproductive health recognises how the call for social and gender justice is driven by, and embedded within, the social and economic contexts through which education is realised.

We were motivated to develop this special issue of *Health Education Journal* by the dual focus on current inequalities and the challenge to ensure good-quality sexual and reproductive health for all. We understand education to take place in both formal and non-formal settings and to include both virtual and digital modes of learning. Digital communication and technologies permit the wider reach of physical and virtual modes of learning as well as the reproduction of inequalities between those who do, and those who do not, have access to them (Arora, 2024). We were particularly interested in how a concern for inclusion, rights and respect could counter the ongoing onslaught on sexual autonomy, and what it might take to restore and advance strategies to deliver on the key goals of quality sexual and reproductive health and education and in particular, universal access to sexual and reproductive health-care services as espoused in the sustainable development goals (Starrs et al., 2024).

Partnerships and allyships are key to transforming poor sexual and reproductive health outcomes. The most effective forms of educational programmes mobilise knowledge skills and values to reinforce rights, restore confidence in people's agency, and strengthen decision-making, while taking account of the social and structural context in which people live their lives (Aggleton et al., 2024).

Partnerships for sexual and reproductive health education

Ongoing debate about the most effective approaches to undertaking sexual and reproductive health education is of significant academic and policy interest. Several different approaches to school-based sexuality education have been documented in the recent literature, reflecting the diversity of cultural, social and political contexts internationally (Chandra-Mouli et al., 2024). However, the pressing question remains: what will work for sexual and reproductive health education in a world marked by inequality, cultural diversity, war and civil conflict, and rapid technological change?

The need for comprehensive sexuality education (CSE) has been emphasised by one critical strand of research (Miedema et al., 2020). CSE goes beyond traditional forms of sex education through its focus on issues of gender, power, consent and rights, all of which are essential for young

people to navigate their sexual and reproductive health responsibly. Research continues to show that CSE is considerably more effective than abstinence-only programmes in reducing rates of sexually transmitted infections and unintended pregnancies among young people (Shepherd et al., 2017). For instance, when implemented with fidelity and adapted to local the context, CSE has led to significant improvements in sexual and reproductive health outcomes, including delayed sexual initiation and increased use of contraception (Wellings et al., 2022). However, CSE may encounter resistance from conservative groups and governments who claim that such an approach runs counter to cultural and religious beliefs. This resistance foregrounds the need for culturally sensitive approaches that engage communities in dialogue rather than imposing external frameworks alongside the false suggestion that these more secular approaches are themselves value-free.

Moreover, debate has increasingly focused on the use of digital platforms and technology in sexual and reproductive health education. The COVID-19 pandemic accelerated the shift towards online learning, creating both opportunities and challenges for sexual and reproductive health education (Evans and Leach, 2023). However, the digital divide remains a significant concern. While new technologies are impacting communities across the world (Arora, 2024), Kickbusch et al. (2021) suggest that in low- and middle-income countries, where Internet access is limited, digital literacy may present challenges to health equity for all. Furthermore, there is a risk that online sexual and reproductive health education may lack the interactive and relational components critical for discussing sensitive topics such as consent and sexual violence (Evans and Leach, 2023). Achieving a balance between technological innovation and human interaction is vital to ensure that sexual and reproductive health education remains equitable and effective.

Another critical issue is the need for an intersectional approach to sexual and reproductive health education especially when addressing the needs of diverse populations, including sex, gender and sexuality diverse youth (Aggleton et al., 2024); people with disabilities (Tarasoff, 2021) and individuals living in conflict-affected areas (Zarocostas, 2024). While engaging with intersectionality is key to achieving beneficial health outcomes (Marshall et al., 2022), a one-size-fits-all approach is insufficient. For example, LGBTQ+ youth frequently face exclusion when sexual and reproductive health education is heteronormative and makes no reference to sexuality and gender diversity and practice. Similarly, girls with disabilities are frequently overlooked by sexual and reproductive health education (McDaniels and Fleming, 2016), despite being at heightened risk of sexual violence and exploitation. Furthermore, race and class inequalities, and associated contextual factors (Bhana et al., 2023), are largely ignored by mainstream sexual and reproductive health education, especially as it relates to the needs of Black women (Bay-Cheng et al., 2019).

In this special issue of *Health Education Journal*, we are motivated by the current state of sexual and reproductive health education – including both the challenges and the potentials. As the papers included in it show, a multifaceted approach that combines comprehensive sexuality education, the thoughtful use of digital tools, and an intersectional lens that ensures inclusivity is necessary. The papers chosen make crystal clear that challenges remain and universal solutions cannot work. What is required is a focus on partnerships and a commitment to the complexity of using a multi-pronged and multi-faceted approach that takes heed of contextual specifics and local realities.

Ultimately, received notions of ‘comprehensiveness’ in CSE which favour menu-driven ‘scientific’ approaches require expansion to engage more fully with culture, context, meaning and values if they are to appeal to, resonate with, and be taken up by politicians and policy makers, national and local education authorities, communities and schools that have hitherto not been engaged in the work that needs to be done.

Beyond controversy: the papers

Given the challenges and the potential that lies in good-quality sexual and reproductive health education, the special issue seeks to move beyond polarised debates, offering insights from different countries and contexts to stress the importance of promoting access to sexual and reproductive health education and rights for all individuals. Our focus on 'Beyond Controversy' stresses the need for inclusive education that respects and engages with diversity, and advocates for a broad-based understanding of what sexual and reproductive health can and should be. Together, the papers in this issue emphasise the need for education to enable people, both young and old, to make informed decisions about their bodies, health, and relationships. While sexual and reproductive health education is fraught with controversy, 'Beyond Controversy' aims to engage these debates by offering evidence-based and inclusive perspectives based on international diversity of experiences and needs in sexual and reproductive health and education.

The articles explore these themes in different ways, including through a focus on the challenges faced by teachers; the need for properly tailored educational resources; the role of peer educators and teachers; and the importance of considering the needs and contributions of marginalised populations such as migrants, people with disabilities and young men grappling with normative expectations around masculinity. The articles within the special issue collectively emphasise the need for inclusive, context-sensitive and responsive educational strategies to address global and local inequalities in sexual and reproductive health education. The careful analysis offered in the articles is complemented by reflections on the implications for policy and practice. As such, we expect this issue to be valuable to researchers, policy-makers and practitioners alike, each paper adding important detail and nuance to ongoing discussion and debate.

This thematic grouping that follows provides structure to the special issue, highlighting the intersections of disability; social-structural and cultural challenges; and gender norms in sexuality education and sexual health.

Inclusive approaches to sexuality education for people with disabilities

Three papers contribute to the theme on 'Inclusive approaches to sexuality education for people with disabilities'. Each of them emphasises the importance of adapting sexuality education to meet the specific needs of people with disabilities, advocating for inclusive and accessible curricula. Together, they highlight barriers such as societal stigmatisation, lack of teacher education and training, and limited access to appropriate teaching materials, all of which hinder effective education for individuals with disabilities. Each paper calls for tailored, evidence-based approaches, including hands-on learning, self-advocacy training and desensitisation for teachers, to empower learners with disabilities to make informed, healthy decisions about their relationships and sexuality.

First, the article 'Stop Leaving People with Disability Behind: Reviewing Comprehensive Sexuality Education for People with Disability' by Kimberley Andreassen, Jordina Quain and Emily Castell, advocates for inclusive approaches to comprehensive sexuality education for people with disabilities. It stresses how current CSE frameworks often neglect issues of accessibility, leaving individuals with disabilities vulnerable to limited self-determination, poor relationship outcomes and sexual abuse. The paper assesses four programmes, noting positive shifts towards inclusion but highlighting ongoing barriers such as funding, insufficient teacher training and the exclusion of topics such as gender identity and sexual diversity. The authors call for the adoption of a rights-based approach that addresses diverse disabilities and emphasises the need for customisable, accessible content that empowers individuals to make informed decisions about relationships, sexual health and personal autonomy. They argue that a comprehensive, inclusive approach

to CSE fosters better health outcomes and inclusion for people with disabilities across various contexts.

The second article 'Adapting a Human Sexuality Curriculum for Students with Disabilities' by Maria Lepore-Stevens reviews the current state of sexuality education for learners with disabilities in the USA. Despite the 1990 'Individuals with Disabilities Education Act', which sought to ensure a free appropriate public education for learners with disabilities, most US states still do not include them in sexuality education curricula. This exclusion leads to heightened risks of unplanned pregnancy, sexual abuse and sexually transmitted infections. The article highlights the damaging effects of societal and physical barriers such as stigmatisation and lack of accessible teaching materials. It advocates for adapted teaching methods, including concrete, hands-on learning, trauma-informed education and caregiver involvement, to support learners' understanding of healthy relationships, consent and self-determination. Best practices emphasise inclusivity, self-advocacy and the use of simple, direct language to cater for diverse learning needs. Recommendations focus on integrating disability-specific content into existing sexuality education frameworks to ensure equitable education for all.

The third article 'Anatomically Correct Models for Visually Impaired Learners: Teachers' Perspectives', by Lindokuhle Ubisi and Wandile Tsabedze, explores South African teachers' attitudes towards the use of anatomically correct models in CSE for visually impaired learners. Through focus group discussions with 36 teachers in five schools for the blind, the study reveals how teachers experience discomfort, embarrassment and confusion about the use of anatomically correct models, often mistaking them for sexual pleasure devices. These reactions stem from a lack of prior training and ingrained cultural and religious norms. Teachers in this study preferred using what they saw as 'neutral' objects like bananas and other fruit rather than anatomically correct models, and expressed concern about the reactions of learners, parents and school administrators were such models to be used. The study emphasises the need for critical teacher education to equip teachers with the confidence and skills to use anatomically correct models effectively as part of CSE lessons. Ultimately, the authors recommend revisions to CSE curricula to better support teachers and learners in addressing sexuality education for visually impaired learners.

Reproductive and menstrual health education

Two papers in the issue address menstrual and reproductive health education, topics of growing concern both nationally and internationally. In the article, 'A Content Analysis of Freely Available Menstrual Education Resources in Australia', Bianca Blackmore addresses the critical yet often overlooked role of menstrual education as part of broader sexual and reproductive health education. While international and national public health agendas prioritise sexual health, menstruation remains a peripheral concern, particularly in Australia. This article examines the extent and depth of menstruation education available beyond mainstream channels, highlighting gaps and limitations in current resources. Through content analysis, the study identifies key themes such as holism, normalisation and the successful management of menstruation, and offers recommendations for more inclusive and comprehensive educational materials that address the biological, psychosocial and cultural aspects of menstrual health.

In their paper, 'Perspectives from the Frontline: Nurses' Experiences of Adolescent Engagement in Sexual and Reproductive Health Services', Jane Kelly and colleagues focus on the critical role nurses play in delivering sexual and reproductive health services to young people in South Africa. The study explores the challenges and opportunities nurses encounter in engaging with young patients, especially in the context of societal stigma, personal biases, and the lasting impact of the

COVID-19 pandemic. Through a qualitative analysis of nurses' experiences, the articles sheds light on the interpersonal dynamics that shape adolescent healthcare outcomes in public health facilities. The study highlights the need for improved communication, non-judgemental care and targeted interventions to enhance the quality of health care offered to adolescents and young people in South Africa.

Sexuality and health education online

The article 'Quality Assessment and Audience Engagement with a Sexuality Education Video: Evidence from China', by Xiaoqin Li, Jiayi Kang and Jiaqi Liang, provides an in-depth analysis of sexuality education videos on the popular Chinese social media platform, Bilibili. Through a content analysis of 203 videos, the study examines the characteristics and quality of these videos and their impact on audience engagement. The findings reveal that reproductive health and sexual pleasure are frequently addressed topics, with non-professional individuals being the primary content creators. The study highlights the positive correlation between video quality and user engagement, suggesting that high-quality videos produced by non-profit organisations attract greater interaction. The authors discuss the implications of this finding for improving the educational design and content accuracy of sexuality education videos on social media, emphasising the potential of such platforms to supplement traditional school-based education in China. The study stresses the need for continued efforts to optimise the quality of sexuality education content in the rapidly evolving digital media landscape.

Contextualising socio-cultural challenges in sexuality education

Three papers focus on the socio-cultural challenges teachers' experience in relation to sexuality education, their role in the provision thereof, relations with broader (school) community and the forms of support these different sets of teachers require. In the first article, 'Teachers navigating the sociocultural challenges of teaching sexuality education to South African learners', Jacqueline Naidoo and Elasmus Muza offer an analysis of Life Orientation (LO) teachers' experiences as they (seek to) traverse the complexities of providing school-based sexuality education in KwaZulu-Natal, South Africa. The authors highlight the strategies deployed by LO teachers to provide young people with the information and skills needed in communities marked by, on one hand, high HIV infection rates and, on the other hand, a reluctance to entrust school teachers with the task of providing sexuality and relationships education. Cultural and religious beliefs and local community practices shaped the language that teachers felt comfortable using and the topics they could openly address. Teachers navigated these tensions and silences by making use of social media material and platforms, and involving specialists and peer teachers. Doing so enabled them to ensure young people acquired the necessary language to talk about their bodies and sex (rather than relying on euphemisms, for example), were exposed to a wider range of 'sensitive' topics, and were provided with evidence-based information. Naidoo and Muza argue for enhanced collaboration across education and health sectors, emphasising the need for proactive support from the Department of Education to assist Life Orientation teachers in implementing culturally sensitive sexuality education strategies effectively.

In the second article, 'Engaging in Emotion and Cultural Work: Migrant Mothers' Experiences of School-Based Sexuality Education', Gabrielle Morina and Smridhi Marwabh utilise the notion of emotion work to make visible the perspectives and experiences of a diverse group of migrant

mothers with respect to their children's school-based sexuality education in French-speaking Québec. As the authors note, emotion work is intangible and difficult to 'measure'. They explore how migrant mothers to Canada navigated their personal discomfort and the importance they attached to their children's education and well-being. The latter involved, among other things, complementing school-based sexuality education with educating children about their socio-cultural heritage. In so doing, the authors highlight how the mothers taking part in the study can support to their children, providing much needed nuance to existing perceptions concerning migrant parents' lack of interest in their children's sexuality education. Attending to mothers' emotion work, the authors seek to contribute to debates on, and the practice of, culturally responsive school-based sexuality education, and specifically, how teachers can better acknowledge and engage with young people's diverse backgrounds.

In the third article, entitled 'Experiences of Peer Teachers in a High School Sexual and Reproductive Health Peer Education Programme in Rwanda', Aimable Nkurunziza and colleagues detail findings from their study of peer teachers' experiences in and of a sexual and reproductive health peer education programme in three public schools in Kirehe district, Rwanda. The programme was developed as part of an effort to reduce teenage pregnancy and in response to the limited impact of prior initiatives on unintended pregnancies among school-going learners. The study revealed the benefits of being a peer educator, such as gaining a sense of increased substantive knowledge and didactic competence, and enhanced relationships with peers. The authors also note that peer educators described some of the challenges they faced, including bullying, and limited time and resources available, as well as strategies used to overcome disruption of their sexuality education initiatives by peers during the initial stages of the programme. Building on the experiences of the peer educators involved, the article offers a series of recommendations for strengthening and scaling up this kind of work. Among other recommendations, the authors note that it is essential that a system is in place to ensure that peer educators are adequately supported in their role, and student needs are met.

Gender norms and sexual health

The precarity of manhood forms the focus of Joseph Vandello, RJ Kubicki and Rebecca Upton's compelling paper 'Young men's sexual health through the lens of precarious manhood'. In many parts of the world, manhood is not a 'developmental certainty', the authors note, but is understood instead as an earned status and one that requires repetition to ensure its stability. Using precarious manhood theory as an organising framework, the authors examine research on young men and women's sexuality and sexual health, arguing that the teenage and emerging adulthood years are times of uncertainty and change for many young men, further heightening concerns about manhood. The paper examines the literature on young men's beliefs and practices with respect to sexual risk-taking and objectification, sexual violence, and sexuality and gender minorities. To give just one example, young men may use sex to prove their manhood, with the fear of sexual failure taking precedence over threats of unintended pregnancy or becoming infected with a sexually transmissible infection. The authors end their paper by offering suggestions for sexual education initiatives, suggesting that considering the precarity of manhood can help support young men in their sexual development. The authors recommend 'gender transformative' approaches and initiatives to support men to critically reflect on the precarity of manhood, challenge gender role norms and explore ideas of manhood that are not or less bound to traditional notions of masculinity, which often emphasise aggression, dominance and sexual conquests.

Conclusion

Despite growing polarisation, growing xenophobia, and anti-gender and anti-feminist backlash in many contexts, the papers in this special issue highlight ongoing efforts to provide good-quality sexual and reproductive health education identifying where there remain opportunities for innovation. Drawing on a wide array of theoretical tools and rich empirical data, the papers contribute to the evidence-base for contextually-relevant approaches to sex, sexuality and relationships education. While all young people have the right to good-quality sexual and reproductive health education, the papers published here highlight the value of dialogue and community engagement, the need for tailored, culturally sensitive approaches, and importance of inclusive curricula that address the specific needs of diverse populations.

What unites the authors whose work is included in this issue is the concern to highlight lesser-known and unheard experiences and perspectives, with inclusivity and intersectionality being two critical lenses across all of the papers. More broadly, the papers examine the complexities of the 'right' to good-quality sexual and reproductive health education for different groups of people (girls and young women, disabled people, young men, gender and sexuality diverse people, migrant mothers, teachers, health workers, and visually impaired learners) at different times and in different places.

Taken together, the papers in this special issue of *Health Education Journal* show how different actors navigate complex emotional, epistemic and political terrains, as well as the significant challenges they encounter in the pursuit of good-quality sexual and reproductive health education. The papers critically examine dominant assumptions regarding the interests and needs of specific groups highlighting how a one-size-fits-all approach to CSE cannot adequately address contextual variations and the specific needs of different groups of people. Instead, the work presented here calls for a more nuanced understanding of sexual and reproductive health education that recognises diversity in young people's and adults' experiences and perspectives. This approach should advocate for the use of context-sensitive practices that go beyond the use of existing frameworks. It should encourage teachers, health care workers and policymakers to rethink how they engage with and implement sexual and reproductive health education in different settings.

Ultimately, we hope the issue will contribute to a growing call to engage in more sensitive and varied ways with sexual and reproductive health education in a world increasingly characterised by ongoing political, religious and socio-cultural controversy and strife.

Funding

The author(s) disclosed receipt of the following financial support for the research, authorship, and/or publication of this article: Deevia Bhana acknowledges the support of the National Research Foundation of South Africa [Grant Number 98407].

References

- Aggleton P, Cover R, Logie CH, et al. (eds) (2024) *Routledge Handbook of Sexuality, Gender, Health and Rights*. 2nd ed. London; New York: Routledge.
- Arora P (2024) The privilege of pessimism: The politics of despair towards the digital and the moral imperative to hope. *Dialogues on Digital Society*. Epub ahead of print 6 May. DOI: 10.1177/29768640241252103.
- Bay-Cheng LY, St Vil NM and Ginn HG (2019) Young women's sexuality in Black and White: Racial differences in appraisals of sexually active young women. *The Journal of Sex Research* 57(3): 296–306.
- Bhana D, Crewe M and Aggleton P (eds) (2023) *Sex, Sexuality and Sexual Health in Southern Africa*. Abingdon and New York: Routledge.

- Chandra-Mouli V, Michielsen K, Gogoi A, et al. (2024) Scaling up, sustaining, and enhancing school-based sexuality education programs in resource constrained and conservative contexts: Replicable lessons from positive-deviant countries. *American Journal of Sexuality Education*. Epub ahead of print 18 August. DOI: 10.1080/15546128.2024.2377071.
- Diniz CSG and Cabral CDS (2022) Reproductive health and rights, and public policies in Brazil: Revisiting challenges during covid-19 pandemics. *Global Public Health* 17(11): 3175–3188.
- Evans C and Leach R (2023) Evaluating the effectiveness of online sexual and reproductive health education: Lessons learned from the COVID-19 pandemic. *Global Public Health* 18(2): 214–229.
- Kickbusch I, Piselli D, Agrawal A, et al. (2021) The Lancet and financial times commission on governing health futures 2030: Growing up in a digital world. *Lancet* 398(10312): 1727–1776
- Marshall Z, Goodyear T and Pike I (2022) Addressing intersectionality in sexual and reproductive health education: A systematic review of the literature. *Sexual and Reproductive Health Matters* 30(1): 154–168.
- McDaniels B and Fleming A (2016) Sexuality education and intellectual disability: Time to address the challenge. *Sexuality and Disability* 34(2): 215–225.
- Miedema E, Le Mat ML and Hague F (2020) But is it comprehensive? Unpacking the ‘comprehensive’ in comprehensive sexuality education. *Health Education Journal* 79(7): 747–762.
- Nash K, O’Malley G, Geoffroy E, et al. (2019) ‘Our girls need to see a path to the future’ – perspectives on sexual and reproductive health information among adolescent girls, guardians, and initiation counselors in Mulanje district, Malawi. *Reproductive Health* 16(1): 8.
- Shepherd LM, Sly KF and Girard JM (2017) Comparison of comprehensive and abstinence-only sexuality education in young African American adolescents. *Journal of Adolescence* 61: 50–63.
- Starrs A, Ezeh A, Sedgh G, et al. (2024) To achieve development goals, advance sexual and reproductive health and rights. *The Lancet* 403(10429): 787–789.
- Tarasoff LA (2021) A call for comprehensive, disability-and LGBTQ-inclusive sexual and reproductive health education. *The Journal of Adolescent Health* 69(2): 185–186.
- Wellings K, Palmer MJ, Geary RS, et al. (2022) Changes in sexual attitudes and lifestyles in Britain through the life course and over time: Findings from the national surveys of sexual attitudes and lifestyles (NATSAL). *The Lancet Global Health* 10(4): e289–e300.
- World Health Organization. (2006). *Defining Sexual Health: Report of a technical consultation on sexual health*. Geneva, Switzerland: World Health Organization.
- Zajacova A and Lawrence EM (2018) The relationship between education and health: Reducing disparities through a contextual approach. *Annual Review of Public Health* 39(1): 273–289.
- Zarocostas J (2024) UNFPA warns of threats to sexual and reproductive health. *The Lancet* 403(10436): 1528.